



Postpartum Depression

Postpartum depression is a type of depression that happens after having a baby. It affects up to 1 in 7 women. You may have emotional highs and lows and frequent crying, or feeling worthless. You may have trouble caring for your baby or yourself. Treatment often involves medication and counseling.

What Is Postpartum Depression?

Postpartum depression (PPD) is a type of **depression** that typically happens after childbirth. If you feel extreme sadness or loneliness, have severe mood swings or anxiety, or lose interest in things you normally enjoy, you may have postpartum depression. You can develop it within the first few weeks of your child's birth or gradually — even up to one year later. But it can also start during pregnancy. Most commonly, it occurs around six weeks **postpartum**.

Postpartum depression doesn't just affect the mother. It can affect surrogates and adoptive parents, too. People experience hormonal, physical, emotional, financial and social changes after having a baby. These changes can make life feel difficult and you may begin to doubt yourself.

If you have postpartum depression, know that you're not alone, it's not your fault and that help is available. Some people never seek treatment out of fear or embarrassment. But your healthcare provider is always a safe space to share your symptoms and feelings. They can help you figure out if you need treatment and what treatment will work best for you.

Stages and severity of postpartum depression

There are three types of postpartum **mood disorders** — baby blues, postpartum depression and postpartum psychosis. The main difference between them is their severity (or intensity) and how long they last.

Postpartum blues or baby blues

Most new moms experience postpartum "baby blues," and this is considered a normal experience. The **baby blues** affect up to 3 in 4 people after delivery. If you're experiencing the baby blues, you may cry for no apparent reason or feel anxious a few times a day. Baby blues typically begin within a few days of delivery, but the symptoms tend to subside within a week or two on their own. If symptoms continue beyond your second week postpartum, contact a healthcare provider.

Postpartum depression

Postpartum depression is more intense and more severe than the baby blues. It can start within one week of delivery and last several months. Symptoms of postpartum depression interfere with your day. Your days tend to feel mostly bad instead of mostly good. You may experience alternating highs and lows, frequent crying, irritability and fatigue. You may also feel that you can't care for your baby or for yourself. Up to 1 in 7 will develop postpartum depression.

Postpartum psychosis

Postpartum psychosis is an extremely severe form of postpartum depression that requires emergency medical attention. This condition is rare, affecting 1 in 1,000 people after delivery. The symptoms

generally come on quickly and intensely, lasting for many weeks or many months. Symptoms include severe agitation, confusion, feelings of hopelessness, delusions or hallucinations, rapid speech and mania.

Symptoms and Causes

Image content: This image is available to view online.

View image online (<https://my.clevelandclinic.org/-/scassets/images/org/health/articles/9312-postpartum-depression>)

Postpartum depression is a type of depression that happens up to one year after having a baby. Contact a healthcare provider if you have signs of postpartum depression like extreme sadness or anxiety and feelings of hopelessness.

How do you know you have postpartum depression?

You may have postpartum depression if you have some of the following:

- Feeling sad, worthless, hopeless or guilty
- Worrying excessively or feeling on edge
- Loss of interest in hobbies or things you usually enjoy
- Changes in appetite or not eating
- Loss of energy and motivation
- Trouble sleeping or wanting to sleep all the time
- Crying for no reason or excessively
- Difficulty thinking or focusing
- Lack of interest in your baby or feeling anxious around your baby

If you have any of the following symptoms, please notify your healthcare provider right away:

- Thoughts of harming yourself or your baby
- Recurrent thoughts of death or suicide
- Depressed mood for most of the day, nearly every day for the last two weeks
- Loss of interest or pleasure in most activities nearly every day for the last two weeks
- Feeling anxious, guilty, hopeless, scared, panicked or worthless nearly every day for the last two weeks

If at any time you feel overwhelmed or you need someone to talk to, call or text 988 to reach the Suicide and Crisis Lifeline (U.S.). Someone is available to help you 24/7.

Having symptoms of postpartum depression doesn't mean you're a terrible parent. Having a baby brings on so many changes and challenges to your life. It's OK to feel overwhelmed or like it's too much for you to handle sometimes. You aren't the first person to feel this way, and you won't be the last.

Contact your healthcare provider if you have symptoms of PPD. An obstetrician, primary care provider and mental health provider are all good options. Your baby's pediatrician can also help you. They can talk to you about possible treatment options.

What causes postpartum depression?

Healthcare providers don't know the exact link between the rapid drop in hormones after delivery and depression, but they know there's a connection. The levels of **estrogen** and **progesterone** increase tenfold during pregnancy but drop sharply after delivery. By three days postpartum, levels of these hormones are back to pre-pregnancy levels.

In addition to these chemical changes, the social and psychological changes of having a baby increase your risk. Examples of these changes include:

- Physical changes to your body
- Lack of sleep and being tired
- Worries about parenting or changes to your relationships

What factors increase my risk of being depressed after the birth of my child?

Certain factors increase your risk for postpartum depression. Some of these are:

- Having a personal or family history of depression, anxiety disorders, postpartum depression with a previous pregnancy or **premenstrual dysphoric disorder (PMDD)**
- Limited social support
- Marital or relationship conflict
- Having doubts about becoming a parent or caring for a child
- Socioeconomic status or worrying about supporting your baby financially
- Pregnancy complications like having a **high-risk pregnancy**, a difficult delivery or **premature birth**
- Being younger than 20 or a single parent
- Having a baby with special needs or a **baby who cries a lot**
- Difficulties with **breastfeeding**
- Challenges adjusting your expectations of parenthood to the reality of parenthood
- Having depression and/or anxiety during pregnancy

What are the complications?

Untreated postpartum depression is dangerous and affects you, your baby and those who love you. Some of the complications of postpartum depression are:

- **Chronic or long-term depression:** This can impact your overall quality of life and put strain on relationships with your partner, family and friends.
- **Being unable to connect with your baby:** Not bonding with your child can affect their development and growth.
- **Increased risk of suicide or thoughts of hurting yourself:** People who don't seek treatment for mental health conditions are more likely to consider suicide. *Please get help immediately if you have thoughts of harming yourself or your baby.*

Can postpartum depression affect my baby?

Yes, postpartum depression can affect your baby.

Research suggests that postpartum depression can affect your baby. Some potential problems are:

- Behavior or learning disorders
- Underdeveloped language and social skills
- Sleep or eating problems
- Higher risk for obesity or developmental delays

Diagnosis and Tests

How is it diagnosed?

There's no specific test that diagnoses postpartum depression. Your healthcare provider will evaluate you at your postpartum visit. This visit may include discussing your health history, how you've felt since delivery, a physical exam, **pelvic exam** and lab tests. Many providers schedule visits at two or three weeks postpartum to screen for depression. This ensures you get the help you need as soon as possible.

They may do a depression screening or ask you a series of questions to assess if you have postpartum depression. They'll ask how you're feeling. Be open and honest with your provider to ensure they get an accurate picture of your emotions and thoughts. They can determine if your feelings are typical or symptoms of postpartum depression.

Your healthcare provider may order a blood test because postpartum depression can cause symptoms similar to many **thyroid conditions**.

Remember, your healthcare provider is there to support you and make sure you're healthy, so be honest with them. There's no judgment, and you aren't alone in your feelings.

How do healthcare providers screen for postpartum depression?

It can be hard to detect mild cases of postpartum depression. Healthcare providers rely heavily on your responses to their questions.

Many healthcare providers use the Edinburgh Postnatal Depression Scale to screen for postpartum depression during pregnancy, as well as during the postpartum period. It consists of 10 questions related to symptoms of depression, such as feeling unhappy, anxious or guilty. You're asked to check the response that comes closest to how you've felt the last seven days. A higher score indicates possible postpartum depression.

If your provider feels you have signs of postpartum depression, they'll recommend an appropriate treatment.

Management and Treatment

How is postpartum depression treated?

Treatment for postpartum depression depends on the type and severity of your symptoms. Treatment options include anti-anxiety or antidepressant medicines, **psychotherapy** (talk therapy or **cognitive behavioral therapy**) and joining support groups. It's also common for your provider to recommend both treatments at the same time.

What medications can I take?

Your healthcare provider may prescribe **antidepressants** to manage symptoms of postpartum depression. Antidepressants help balance the chemicals in your brain that affect your mood. You can take antidepressant medication during pregnancy, delivery and into the postpartum period.

If you're breastfeeding, talk to your healthcare provider about the risks and benefits of taking an antidepressant. Medications can transfer to your baby through your breast milk. But the transfer level is generally low, and many antidepressant medications are safe. Your provider can help you decide which medicine is right for you based on your symptoms and whether you're nursing.

Some common antidepressants for postpartum depression are:

- Selective serotonin reuptake inhibitors (SSRIs), such as **sertraline** (Zoloft[®]) and **fluoxetine** (Prozac[®])
- Serotonin and norepinephrine reuptake inhibitors (SNRIs), such as **duloxetine** (Cymbalta[®]) and **desvenlafaxine** (Pristiq[®])
- **Bupropion** (Wellbutrin[®] or Zyban[®])

- Tricyclic antidepressants (TCAs), such as **amitriptyline** (Elavil®) or **imipramine** (Tofranil®)

Keep in mind that it takes at least two or four weeks for antidepressants to work completely. Talk to your healthcare provider before stopping the medication. Stopping your medication too soon can cause symptoms to return. You should stay on your antidepressant for at least six months to one year before tapering off. Most providers will recommend reducing your dose before stopping completely.

If your provider detects postpartum depression while you're still in the hospital, they may recommend IV medication containing **brexanolone**.

What are ways to cope?

It's OK to feel overwhelmed. Parenting is full of ups and downs, and having a baby isn't easy. If you have depression, you don't have to accept your feelings as normal or try to get over them by yourself. Don't be embarrassed to share your feelings with people close to you. Your healthcare provider can help find a treatment that works for you.

Here are some things you can do to help cope with postpartum depression:

- Find someone to talk to — a therapist, friend, family member or someone who will listen to you and help you.
- Try to get help early on with your depression and/or anxiety, especially if you're having symptoms during pregnancy. Don't delay. Symptoms will likely worsen during the postpartum period without treatment.
- If breastfeeding is too stressful, prioritize your mental health and make the decision to bottle-feed. This allows for more flexibility in having others feed your baby.
- Join a support group for new parents.
- Try to eat as healthily as you can and find time for exercise.
- Prioritize rest for yourself.
- Get fresh air when you can. Being outside can make you feel better.
- Find time for self-care and doing things you enjoy, like reading or other hobbies.
- Get help with household chores or errands.

How long does postpartum depression last?

Postpartum depression can last until one year after your child is born. But this doesn't mean you should feel or be "cured" in one year. Talk to your healthcare provider about your symptoms and treatment. Be honest about how you feel. Think carefully about whether you feel better than you did at the beginning of your diagnosis. Then, they can recommend ongoing treatment for your symptoms.

Does PPD go away by itself?

Not usually. Most people need some type of treatment to feel better. Not everyone needs the same treatment, and not everyone feels relief at the same time or in the same way. But most healthcare providers agree that some intervention is necessary to help with symptoms of postpartum depression.

Outlook / Prognosis

What can I expect if I have postpartum depression?

The reality of being a new parent can hit you hard. You may have expected life with a baby to be mostly rainbows and butterflies, but instead you feel disconnected, lonely and unhappy. You may ask yourself, "Am I doing this right?" or "Is it normal to feel like this?" You're not alone in your feelings. Hundreds of thousands of people have been in your shoes. And with professional help, many of them have stepped out of those shoes, overcoming the challenges and symptoms postpartum depression can bring.

The first step is seeking help. You might feel shame, embarrassment or guilt for taking care of yourself when you're trying to take care of your newborn. Postpartum depression isn't something that magically goes away on its own. It's a real condition, and most people will get better with treatment. There can be complications if you don't get help. Remember, seeking help isn't a sign of weakness.

Once you and your healthcare provider determine the best treatment, stick with your treatment plan. If your treatment involves medication, take it exactly as your provider prescribes. Don't attempt to increase or decrease your dosage on your own. If your provider thinks you'll benefit from counseling, try it for several sessions to see if it helps you feel better. You can also try attending virtual support groups.

Postpartum depression isn't your fault. There are resources available to help relieve your symptoms and get you back to feeling like yourself.

Prevention

Can postpartum depression be prevented?

Postpartum depression isn't entirely preventable. It helps to know the warning signs of the condition and the factors that increase your risk. Here are some tips that can help prevent postpartum depression:

- Be realistic about your expectations for yourself and your baby.
- Limit visitors when you first go home.
- Ask for help — let others know how they can help you.
- Sleep or rest when your baby sleeps.
- Exercise — take a walk and get out of the house for a break.
- Keep in touch with your family and friends — don't isolate yourself.
- Foster your relationship with your partner — make time for each other.
- Expect some good days and some bad days.

Living With

When should I seek professional medical treatment for symptoms related to postpartum depression?

You should seek professional help when:

- Symptoms of depression persist beyond two weeks
- You can't cope with everyday situations or make seemingly easy decisions
- You feel extremely anxious, scared and panicked most of the day
- You cry for no apparent reason several times each day

For immediate help, or if you have thoughts of hurting yourself or someone else:

- **Dial 911 in an emergency.**
- **Call or text the Suicide and Crisis Lifeline at 988 or use their chat feature online. They can also provide free and confidential emotional support.**
- **Free resources are available through Postpartum Support International. Call the PSI HelpLine at 1-800-944-4773.**

Additional Common Questions

How can I help someone with postpartum depression?

People with postpartum depression need lots of support. Here are some ways you can help:

- Know the signs of depression and anxiety, and urge your friend or partner to seek medical care.
- Be a good listener. Let them know you're there to listen and help.
- Offer to help them with daily tasks like cleaning, making meals and running errands.
- Offer to help watch their baby while they sleep or rest.

If you know someone with symptoms of PPD, offer to call their provider for them. They may feel shame or be afraid to ask for help, or they may not realize how severe postpartum depression can be. You could also go with them to their appointment as a support person.

A note from Cleveland Clinic

Postpartum depression can happen to anyone. Having a new baby to care for — while also caring for yourself and other loved ones — can be overwhelming, to say the least. The biological, physical and social factors that cause postpartum depression are beyond your control. It's not your fault, and you did nothing to cause it. It doesn't make you a bad parent, and you're not the first person to feel the way you feel.

Everyone has good days and bad days. But if you feel like you have more bad days than good days, it's time to reach out to your healthcare provider for help. They can figure out the best way to manage your symptoms. Counseling, medication and joining a support group are good ways to help you cope with PPD and get you feeling better.

Experts You Can Trust

✓ **Medically Reviewed.** Last updated on 03/26/2026.

References

Cleveland Clinic's health articles are based on evidence-backed information and review by medical professionals to ensure accuracy, reliability and up-to-date clinical standards.

- American Academy of Family Physicians. Postpartum Depression (PPD) (<https://familydoctor.org/condition/postpartum-depression/>). Last updated 2/2024. Accessed 3/26/2026.
- American Psychiatric Association. What is Perinatal Depression? (<https://www.psychiatry.org/patients-families/postpartum-depression/what-is-postpartum-depression>) Last reviewed 10/2023. Accessed 3/26/2026.
- American Psychological Association. Postpartum depression: Causes, symptoms, risk factors, and treatment options (<https://www.apa.org/topics/women-girls/postpartum-depression>). Last updated 11/2022. Accessed 3/26/2026.
- Green TL, Son YK, Simuzingili M, Mezuk B, et al. Pregnancy-Related Weight and Postpartum Depressive Symptoms: Do the Relationships Differ by Race/Ethnicity? (<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC10163440/>) *J Womens Health (Larchmt)*. 2021 Jun;30(6):816-828. Accessed 6/24/2024.
- Mughal S, Azhar Y, Siddiqui W. Postpartum Depression (<https://www.ncbi.nlm.nih.gov/books/NBK519070/>). 2025 Jan 22. In: *StatPearls* [Internet]. Treasure Island (FL): StatPearls Publishing; 2026 Jan. Accessed 3/26/2026.
- National Institute of Mental Health. Perinatal depression (<https://www.nimh.nih.gov/health/publications/perinatal-depression>). Last revision 6/2023. Accessed 3/26/2026.
- Office on Women's Health (U.S.). Postpartum depression (<https://www.womenshealth.gov/mental-health/mental-health-conditions/postpartum-depression>). Last updated 10/2023. Accessed 3/26/2026.
- Stewart DE, Vigod SN. Postpartum Depression: Pathophysiology, Treatment, and Emerging Therapeutics. (<https://pubmed.ncbi.nlm.nih.gov/30691372/>) *Annu Rev Med*. 2019 Jan 27;70:183-196. Accessed 3/26/2026.
- The American College of Obstetricians and Gynecologists. Postpartum depression (<https://www.acog.org/womens-health/faqs/postpartum-depression>). Last reviewed 12/2021. Accessed 3/26/2026.

- The American College of Obstetricians and Gynecologists. Practice Advisory: August 2023. Zuranolone for the Treatment of Postpartum Depression (<https://www.acog.org/clinical/clinical-guidance/practice-advisory/articles/2023/08/zuranolone-for-the-treatment-of-postpartum-depression>). Last updated 1/2024. Accessed 3/26/2026.
- U.S. Preventative Services Task Force. Recommendation Statement: Perinatal Depression Prevention Interventions (<https://www.uspreventiveservicestaskforce.org/uspstf/recommendation/perinatal-depression-preventive-interventions#fullrecommendationstart>). Last updated 2/2019. Accessed 3/26/2026.

Call: 800.223.2273 | 9500 Euclid Avenue, Cleveland, Ohio 44195 | © **2026 Cleveland Clinic**. All Rights Reserved.